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Time taken	11 secs
Marks	0/42
Grade	0 out of 100

Question 1

Not answered

Marked out of 1

Which of the following is an established risk factor for experiencing depression during pregnancy?

Select one:

- ☐ Smoking cigarettes
- ☐ Early age at menarche
- ☐ Past history of depression
- ☐ First pregnancy
- ☐ Working mother

Your answer is incorrect.

Explanation:

Risk factors for experiencing depression during pregnancy include a family or personal history of depression; ambivalence about the pregnancy; and lack of marital, family, or social supports.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 492.

The correct answer is: Past history of depression

Question 2

Not answered

Marked out of 1

Which drug is widely used by perinatal services in the UK for the treatment of schizophrenia in pregnancy?

Select one:

- ☐ Olanzapine
- ☐ Amisulpride
- ☐ Quetiapine
- ☐ Risperidone
- ☐ Aripiprazole

Your answer is incorrect.

Risk associated with untreated schizophrenia is high. The current consensus is to use an antipsychotic agent at every stage of pregnancy. Patients with a history of psychosis, who are maintained on antipsychotic, particularly if they have frequent relapses, are best maintained on antipsychotic during and after pregnancy. Most experience is with chlorpromazine, trifluoperazine, haloperidol, olanzapine and clozapine. Olanzapine is widely used by perinatal services in the UK.

The correct answer is: Olanzapine

Question 3

Not answered

Marked out of 1

Which of the following is not an evidence-based treatment for premenstrual dysphoric disorder (PMDD)?

Select one:

- ☐ Clomipramine
- ☐ Venlafaxine
- ☐ Sertraline
- ☐ Fluoxetine
- ☐ Reboxetine

Your answer is incorrect.

Explanation:

Selective serotonin reuptake inhibitors (SSRIs) are considered first-line treatment in premenstrual dysphoric disorder (PMDD). SSRIs may be taken continuously, for the full premenstrual phase, or as 'symptom-onset' dosing. Studies with serotonin norepinephrine reuptake inhibitors (SNRIs) have been fewer, but studies of venlafaxine or duloxetine have shown benefit. RCTs have also reported efficacy with clomipramine, a tricyclic antidepressant with largely serotonergic action, in daily dosing and luteal phase dosing. In sum, antidepressant treatments for PMDD include fluoxetine, sertraline, paroxetine, citalopram, escitalopram, venlafaxine, and clomipramine.

Ref: di Scalea TL, Pearlstein T. Premenstrual dysphoric disorder. Med Clin North Am. 2019 Jul;103(4):613-628.

The correct answer is: Reboxetine

Question 4

Not answered

Marked out of 1

To screen for Ebstein's anomaly, pregnant mothers should undergo level 2 ultrasound and echocardiography of the foetus at

Select one:

- ☐ 2 and 10 weeks gestation
- ☐ 2 and 6 weeks gestation
- ☐ 24 and 32 weeks gestation
- ☐ 6 and 18 weeks gestation
- ☐ 20 and 24 weeks gestation

Your answer is incorrect.

To screen for Ebstein's anomaly, mothers should undergo level 2 ultrasound and echocardiography of the foetus at 6 and 18 week's gestation. (Maudsley Guidelines 2007). For women on maintenance treatment, the serum lithium level should be monitored every 4 weeks throughout the pregnancy. Lithium dosage should be adjusted to match the lower end of the therapeutic range. Lower doses and frequent blood monitoring should be the norm in pregnant women starting lithium in the first trimester of pregnancy. Lithium commenced in second and third trimester of pregnancy or perinatal period can help reduce the risk of puerperal psychosis.

The correct answer is: 6 and 18 weeks gestation

Question 5

Not answered

Marked out of 1

What is the chance of having a baby with congenital malformation if lithium is continued through the first trimester?

Select one:

- ☐ 1 in 10
- ☐ 1 in 1000
- ☐ 1 in 2
- ☐ 1 in 100
- ☐ 1 in 5

Your answer is incorrect.

There is 1 in 10 chance of having a malformation if lithium is continued through the first trimester. The UK National Teratology Information Service have concluded that lithium increases the risk of all types of malformation of approximately three-fold and with a weighting towards cardiac malformations of around eight-fold (Williams and Oke, 2000). Maximum risk is at 2-6 weeks after conception when many pregnancies are still undiscovered.

The correct answer is: 1 in 10

Question 6

Not answered

Marked out of 1

The most sensitive period for foetal malformations on exposure to teratogens is

Select one:

- ☐ 32-36 weeks
- ☐ At labour
- ☐ 14-28 weeks
- ☐ 2-14 weeks
- ☐ 28-32 weeks

Your answer is incorrect.

Explanation:

The most sensitive period for foetal malformations on exposure to teratogens is the first trimester.

Ref: National Institute for Health and Clinical Excellence. Antenatal and postnatal mental health: clinical management and service guide. Updated edition 2020.

The correct answer is: 2-14 weeks



Question 7

Not answered

Marked out of 1

Mrs. X has a history of postpartum psychosis, which occurred two years ago. She is pregnant again and would like to know the risk of recurrence rate in subsequent pregnancies. The correct answer is

Select one:

- ☐ 0.1-0.25%
- ☐ 5%
- ☐ 10%
- ☐ 50-90%
- ☐ 25-40%

Your answer is incorrect.

The risk of postpartum psychosis is 0.1-0.25% in general population, 50% in bipolar disorder and 50-90% in a patient with a history of postpartum psychosis. Puerperal psychosis can take the form of mania, schizophreniform / acute polymorphic psychosis, or with confusion and disorientation. The incidence of puerperal psychosis is about one per 1000 births. It is strongly linked with bipolar disorder.

The correct answer is: 50-90%



Question 8

Not answered

Marked out of 1

The prevalence of perinatal depression in women is

Select one:

- ☐ 30%
- ☐ 20%
- ☐ 10%
- ☐ 25%
- ☐ 5%

Your answer is incorrect.

Explanation:

The prevalence of perinatal depression in women is 10-15%. A 10% risk of significant depression is seen in the first trimester, associated with a history of depression, abortion, intrauterine loss, or unwanted pregnancy.

Ref: Yang K. et al. Risk factors of perinatal depression in women: a systematic review and meta-analysis. BMC psychiatry. 2022 Dec;22(1):1-11.

Sample D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 494.

The correct answer is: 10%

Question 9

Not answered

Marked out of 1

Postpartum psychosis is most commonly associated with

Select one:

- ☐ hysteria
- ☐ delusional disorder
- ☐ delusional jealousy
- ☐ schizophrenia
- ☐ bipolar disorder

Your answer is incorrect.

Explanation:

Psychotic episodes are more prevalent during the postpartum period than during other periods in a woman's life, and evidence clearly suggests a particular vulnerability in women with bipolar disorder. Women with bipolar disorder have the highest risk of postpartum psychosis compared with women with other psychiatric diagnoses. Women with postpartum psychosis often have a history of bipolar disorder.

Ref: Meltzer-Brody S et al. Postpartum psychiatric disorders. Nature Reviews Disease Primers. 2018 Apr 26;4(1):1-8.

The correct answer is: bipolar disorder



Question 10

Not answered

Marked out of 1

A 35-year-old woman has a history of depression, currently euthymic in mood. She is now six weeks pregnant. If she stops medication now, what is the possibility of risk of relapse?

Select one:

- ☐ No risk of relapse
- ☐ 2 times
- ☐ 10 times
- ☐ 3 times
- ☐ 5 times

Your answer is incorrect.

The chance of relapse in pregnant mothers who take antidepressants depends on the number of previous episodes and pregnant women who stop taking antidepressants are five times more likely to experience a depression relapse than are pregnant women who continue taking the drugs

The correct answer is: 5 times



Question 11

Not answered

Marked out of 1

Which of the following effects on the fetus is NOT associated with a mother being treated with SSRIs during pregnancy?

Select one:

- ☐ Neonatal irritability
- ☐ Spontaneous abortion
- ☐ Premature birth
- ☐ Neural tube defects
- ☐ Reduced birth weight

Your answer is incorrect.

SSRI: No increase in major malformation (exception- paroxetine); the neonatal withdrawal are reversible complications. However there is 13.3% increase in spontaneous abortion (also with mirtazepine and amp; bupropion), decreased gestational age (mean 1 week) and low birth weight (mean 175 gms). Paroxetine, particularly high dose first trimesters exposure, is clearly linked to cardiac malformation VSD and ASD. Third trimester use can give rise to neonatal complication due to abrupt withdrawals.

The correct answer is: Neural tube defects

Question 12

Not answered

Marked out of 1

Mrs. Y has a history of bipolar disorder and remained on mood stabilizers whilst pregnant. Following delivery, the breastfed infant suffered from thrombocytopenia and anaemia which resolved after stopping breastfeeding. The most likely offending drug is

Select one:

- ☐ Gabapentin
- ☐ Lamotrigine
- ☐ Lithium
- ☐ Sodium valproate
- ☐ Carbamazepine

Your answer is incorrect.

Sodium valproate achieves a serum range in breastfed infants that varies from being undetectable to up to 40%. Adverse effects reported include thrombocytopenia and anaemia. These features resolve after stopping breast feeding.

The correct answer is: Sodium valproate



Question 13

Not answered

Marked out of 1

If a patient continues to take valproate throughout her pregnancy, what is the risk of the baby having a major malformation?

Select one:

- ☐ 1 in 1000
- ☐ 1 in 10
- ☐ 1 in 10000
- ☐ 1 in 100 000
- ☐ 1 in 100

Your answer is incorrect.

Explanation:

Valproate has a clear causal link with increased risk of foetal abnormalities, particularly neural tube defects including spina bifida. Valproate confers a high risk – around 10% for major malformations - and should not be used in women of child-bearing age except where all other treatment has failed.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 690-691.

The correct answer is: 1 in 10

Question **14**

Not answered

Marked out of 1

If a patient continues to take valproate throughout her pregnancy, what is the risk of the baby having a neural tube defect (spina bifida)?

Select one:

- ☐ 1 in 10
- ☐ 1 in 1000
- ☐ 1 in 10 000
- ☐ 1 in 100 000
- ☐ 1 in 100

Your answer is incorrect.

Explanation:

Valproate is associated with a significant increased risk of a range of major congenital malformations, particularly neural tube defects. Significant associations have been reported for spina bifida, with an approximate 13-fold increased risk from approximately 0.09% in the general population to 1-2% with valproate.

Ref: McAllister-Williams RH et al. British Association for Psychopharmacology consensus guidance on the use of psychotropic medication preconception, in pregnancy and postpartum 2017. Journal of Psychopharmacology. 2017 May;31(5):519-52.

The correct answer is: 1 in 100



Question 15

Not answered

Marked out of 1

With respect to postpartum mental illness, a significant increase in new psychiatric episodes exists in the first

Select one:

- ☐ 1 week postpartum
- ☐ 12 months postpartum
- ☐ 6 months postpartum
- ☐ 3 months postpartum
- ☐ 1 day postpartum

Your answer is incorrect.

Explanation:

In particular, an increased risk of the onset or worsening of psychiatric illness, including mood disorders, anxiety disorders and psychosis, exists during the first 3 months postpartum.

Ref: Meltzer-Brody S et al. Postpartum psychiatric disorders. Nature Reviews Disease Primers. 2018 Apr 26;4(1):1-8.

The correct answer is: 3 months postpartum



Question 16

Not answered

Marked out of 1

What percentage of women of reproductive age are affected by premenstrual dysphoric disorder?

Select one:

- ☐ 25-30%
- ☐ 0.5-1%
- ☐ 15-20%
- ☐ 80-90%
- ☐ 2-8%

Your answer is incorrect.

Explanation:

About 85% of women experience at least one mild premenstrual symptom. 20-25% experience moderate-severe premenstrual symptoms (premenstrual syndrome or PMS). Premenstrual dysphoric disorder (PMDD) is characterised by psychological and somatic symptoms as well as functional impairment that lie on the severe end of the continuum of premenstrual symptoms. PMDD is now included in DSM-5 and ICD-11. PMDD occurs in 2-8% of women with regular menstrual cycles.

Ref: di Scalea TL, Pearlstein T. Premenstrual dysphoric disorder. Med Clin North Am. 2019 Jul;103(4):613-628.

Sample D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 488.

The correct answer is: 2-8%

Question 17

Not answered

Marked out of 1

Which of the following has the largest evidence base for the treatment of severe premenstrual syndrome and premenstrual dysphoric disorder?

Select one:

- ☐ Alprazolam
- ☐ Primrose oil
- ☐ Citalopram
- ☐ Fluoxetine
- ☐ Amitriptyline

Your answer is incorrect.

Explanation:

An SSRI is considered first-line treatment for severe premenstrual syndrome (PMS) and premenstrual dysphoric disorder (PMDD). Fluoxetine is the most studied.

Ref: di Scalea TL, Pearlstein T. Premenstrual dysphoric disorder. Med Clin North Am. 2019 Jul;103(4):613-628.

Sample D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 491.

Carlini SV, Deligiannidis KM. Evidence-based treatment of premenstrual dysphoric disorder: a concise review. The Journal of clinical psychiatry. 2020 Feb 4;81(2):6789.

The correct answer is: Fluoxetine

Question **18**

Not answered

Marked out of 1

What is the relative risk of developing Ebstein's anomaly for a child when the mother is on lithium throughout her pregnancy?

Select one:

- ☐ 3.5
- ☐ 2.5
- ☐ 5.5
- ☐ 7.4
- ☐ 9.5

Your answer is incorrect.

Relative risk of Ebstein's anomaly in mothers taking lithium compared to general population is 10-20times higher, but the absolute risk is relatively low at 1:1000. (Absolute spontaneous risk of Ebsteins is 1 in 20,000. Cohen et al., JAMA 1994;271: 146-150). Maximum risk is seen at 2-6 weeks after conception when many pregnancies are still undiscovered. The closest answer from the list is - 9.5 times.

The correct answer is: 9.5



Question 19

Not answered

Marked out of 1

Which of the following psychotropic medications is not recommended for initiation in breastfeeding mothers?

Select one:

- ☐ Sertraline
- ☐ Lorazepam
- ☐ Clonazepam
- ☐ Mirtazapine
- ☐ Olanzapine

Your answer is incorrect.

Explanation:

When initiating an antidepressant during breastfeeding, sertraline and mirtazapine may be considered. When initiating an antipsychotic, olanzapine or quetiapine may be considered. Sedatives are best avoided, but when used during breastfeeding, a drug with a short half-life should be used. Lorazepam may be considered.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 703.

The correct answer is: Clonazepam



Question 20

Not answered

Marked out of 1

Following childbirth, what percentage of women develop postpartum depression?

Select one:

- ☐ 25-30%
- ☐ 65-70%
- ☐ 45-50%
- ☐ 1-2%
- ☐ 10-15%

Your answer is incorrect.

Explanation:

A significant depressive episode temporally related to childbirth occurs in 10-15% of women within 6 months postpartum (peak 3-4 weeks). Risk factors include a personal/family history of depression, older age, being a single mother, having a poor relationship with own mother, ambivalence towards or unwanted pregnancy, poor social support, additional psychosocial stressors, severe 'baby blues', and previous postpartum psychosis.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 494.

The correct answer is: 10-15%

Question **21**

Not answered

Marked out of 1

Up to what percentage of new mothers will experience 'baby blues'?

Select one:

- ☐ 20%
- ☐ 10%
- ☐ 95%
- ☐ 2%
- ☐ 75%

Your answer is incorrect.

Explanation:

'Baby blues' are usually described as transient, mild mood and anxiety symptoms that often persist for up to two weeks and usually resolve spontaneously with no sequelae. Up to three-quarters (75%) of new mothers will experience this short-lived period of tearfulness and emotional lability.

Ref: Meltzer-Brody S et al. Postpartum psychiatric disorders. Nature Reviews Disease Primers. 2018 Apr 26;4(1):1-8.

Sample D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 494.

The correct answer is: 75%

Question 22

Not answered

Marked out of 1

Of the following, the least likely risk factor for postpartum depression is

Select one:

- ☐ Personal history of postpartum depression
- ☐ First degree relative with bipolar disorder
- ☐ First degree relative with postpartum depression
- ☐ Mild baby blues after delivery
- ☐ Personal history of bipolar disorder

Your answer is incorrect.

Explanation:

A significant depressive episode temporally related to childbirth occurs in 10-15% of women within 6 months postpartum (peak 3-4 weeks). Risk factors include a personal/family history of depression, older age, being a single mother, having a poor relationship with own mother, ambivalence towards or unwanted pregnancy, poor social support, additional psychosocial stressors, severe 'baby blues', and previous postpartum psychosis. Women with bipolar disorder have a high risk of postpartum episodes, including depression, anxiety, mania, and psychosis.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 494.

Meltzer-Brody S et al. Postpartum psychiatric disorders. Nature Reviews Disease Primers. 2018 Apr 26;4(1):1-8.

The correct answer is: Mild baby blues after delivery

Question **23**

Not answered

Marked out of 1

The most severe and early symptoms seen in premenstrual syndrome are

Select one:

- ☐ Anxiety and Irritability
- ☐ Depressed mood and anhedonia
- ☐ Breast tenderness and bloating
- ☐ Insomnia and poor appetite
- ☐ Concentration difficulties and memory loss

Your answer is incorrect.

Symptoms of PMS peak 2 days before start of menses and last for few days to 2 weeks. The most severe symptoms are anger and irritability, which start earlier than other symptoms. Women with PMS tend to have the same symptoms from one cycle to the next. Severe PMS affects 3-8% women in reproductive age. Comorbidity with mood disorder is seen in 30-70%. The risk of developing premenstrual depression and postnatal depression are higher in women with PMS.

The correct answer is: Anxiety and Irritability

Question **24**

Not answered

Marked out of 1

What proportion of women with a psychiatric history of recurrent depression are likely to relapse when antidepressants are discontinued during pregnancy?

Select one:

- ☐ 90%
- ☐ 45%
- ☐ 70%
- ☐ 10%
- ☐ 25%

Your answer is incorrect.

Explanation:

Findings on the risk of relapse of unipolar depression if medication is stopped during or prior to pregnancy are inconsistent, possibly depending on the severity of illness. Very high rates of relapse of depression (70%) have been found in some studies when women have discontinued antidepressant medication, but these studies were conducted in psychiatric settings and recruited women with histories of recurrent depression. In obstetric settings, likely to include women with milder forms of depression, the risk of a major depressive episode has been shown to be similar whether or not women continued antidepressant medication. Risk is likely to be highest for women with a history of severe and/or recurrent depression.

Ref: McAllister-Williams RH et al. British Association for Psychopharmacology consensus guidance on the use of psychotropic medication preconception, in pregnancy and postpartum 2017. Journal of Psychopharmacology. 2017 May;31(5):519-52.

Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 685.

The correct answer is: 70%

Question 25

Not answered

Marked out of 1

Which of the following is not a feature of premenstrual dysphoric disorder, according to DSM-5 diagnostic criteria?

Select one:

- ☐ Irritability
- ☐ Feelings of guilt
- ☐ Depressed mood
- ☐ Anxiety
- ☐ Decreased interest in usual activities

Your answer is incorrect.

Explanation:

According to DSM-5, a minimum of 5 symptoms must be present for the diagnosis of premenstrual dysphoric disorder (PMDD), including one 'core' symptom: marked affective lability, irritability, depressed mood, or anxiety. Other potential PMDD symptoms include decreased interest in usual activities, difficulty concentrating, low energy, changes in sleep or appetite, sense of being overwhelmed or out of control, and physical symptoms.

Ref: American Psychiatric Association. Diagnostic and statistical manual of mental disorders (DSM-5®). American Psychiatric Pub; 2013 May 22.

di Scalea TL, Pearlstein T. Premenstrual dysphoric disorder. Med Clin North Am. 2019 Jul;103(4):613-628.

The correct answer is: Feelings of guilt

Question **26**

Not answered

Marked out of 1

Since delivering a baby two weeks ago, a 32-year-old woman has been very tearful, low, and avoiding food. She spends most nights awake in a fearful, anxious state and is finding it difficult to breastfeed, believing that her breasts are involuting. The most likely diagnosis is

Select one:

- ☐ Postpartum depression
- ☐ Adjustment disorder
- ☐ Dissociative neurological symptom disorder
- ☐ Baby blues
- ☐ Postpartum psychosis

Your answer is incorrect.

Explanation:

Postpartum psychosis is usually a rapid onset, severe psychosis, typically starting within the first 2-4 weeks after childbirth, and is considered a psychiatric emergency as a lack of self-care, and an inability to care for the infant can lead to suicide and/or, in rare cases, infanticide. Symptoms can manifest as delusions, hallucinations, and, particularly, confusion and perplexity, and patients can also have severe mood swings, insomnia, agitation, and rapid deterioration.

Ref: Meltzer-Brody S et al. Postpartum psychiatric disorders. Nature Reviews Disease Primers. 2018 Apr 26;4(1):1-8.

The correct answer is: Postpartum psychosis

Question **27**

Not answered

Marked out of 1

Of those listed, the antidepressant with the most favourable evidence for use in postpartum breastfeeding mothers is

Select one:

- ☐ Paroxetine
- ☐ Desipramine
- ☐ Sertraline
- ☐ Trazodone
- ☐ Duloxetine

Your answer is incorrect.

Explanation:

When initiating an antidepressant during breastfeeding, sertraline and mirtazapine may be considered. SSRIs with lowest levels of exposure or adverse physical effects in breastfeeding include sertraline and paroxetine. Given the greater risk of side effects and withdrawal problems with paroxetine compared with other SSRIs, sertraline would be the preferred treatment option. Plasma concentrations of sertraline in breastfeeding infants are undetectable/low.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 703, 708.

McAllister-Williams RH et al. British Association for Psychopharmacology consensus guidance on the use of psychotropic medication preconception, in pregnancy and postpartum 2017. Journal of Psychopharmacology. 2017 May;31(5):519-52.

The correct answer is: Sertraline

Question 28

Not answered

Marked out of 1

Which of the following neurotransmitters most likely has an aetiological role in the pathogenesis of premenstrual syndrome?

Select one:

- ☐ Noradrenaline
- ☐ Dopamine
- ☐ Adrenaline
- ☐ Serotonin
- ☐ Acetylcholine

Your answer is incorrect.

Explanation:

The role of serotonin in the cause of premenstrual syndrome/premenstrual dysphoric disorder (PMS/PMDD) is supported by numerous findings using indirect measures of central serotonin and its transmission, and by probes of the serotonin system. Studies include reduced whole blood serotonin concentration during the luteal phase, reduced serotonin and imipramine binding at the platelet plasma membrane serotonin transporter, exacerbation of premenstrual irritability and anxiety after tryptophan depletion, delayed plasma prolactin response to fenfluramine, and blunted plasma cortisol and ACTH responses to the serotonergic agent m-chlorophenylpiperazine.

Ref: di Scalea TL, Pearlstein T. Premenstrual dysphoric disorder. Med Clin North Am. 2019 Jul;103(4):613-628.

The correct answer is: Serotonin

Question **29**

Not answered

Marked out of 1

A 31-year-old mother of a 3-week-old baby scores high (>16) on the Edinburgh Depression Scale. This increases the risk of

Select one:

- ☐ delirium
- ☐ spousal violence
- ☐ suicidal ideation
- ☐ harm to the baby
- ☐ psychotic symptoms

Your answer is incorrect.

Explanation:

The Edinburgh Postnatal Depression Scale is a 10-item tool developed for screening postpartum women for depression in outpatient clinics, home visits, or during postpartum appointments. It contains dimensions of depression, anxiety, anhedonia, and self-harm/suicide.

Ref: Smith-Nielsen J et al. Validation of the Edinburgh Postnatal Depression Scale against both DSM-5 and ICD-10 diagnostic criteria for depression. BMC psychiatry. 2018 Dec;18(1):1-2.

The correct answer is: suicidal ideation

Question 30

Not answered

Marked out of 1

If carbamazepine is used during pregnancy, what prophylactic drug should be given to the mother and neonate after delivery?

Select one:

- ☐ Vitamin A
- ☐ Vitamin D
- ☐ Thyroid supplements
- ☐ Vitamin K
- ☐ Thiamine

Your answer is incorrect.

Explanation:

NICE advises discussing the possibility of stopping carbamazepine if a woman is planning a pregnancy or becomes pregnant. If carbamazepine is used, prophylactic vitamin K should be administered to the mother and neonate after delivery.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 692.

The correct answer is: Vitamin K



Question **31**

Not answered

Marked out of 1

In women who have a history of recurrent major depression, what percentage of pregnancies result in relapse?

Select one:

- ☐ 15%
- ☐ 40%
- ☐ 7%
- ☐ 1%
- ☐ 25%

Your answer is incorrect.

Explanation:

A personal history of recurrent major depression is a very strong risk factor for the occurrence of perinatal depression: around 40% of pregnancies result in relapse.

Ref: Doyle M et al. Perinatal depression and psychosis: an update. BJPsych Advances. 2015 Jan;21(1):5-14.

The correct answer is: 40%



Question **32**

Not answered

Marked out of 1

A 31-year-old pregnant woman declines pharmacotherapy for her depression. She has severe depressive features and suicidal risk. Which of the following factors is most relevant if she is considered for ECT?

Select one:

- ☐ Increased protein binding in pregnancy
- ☐ Decreased protein binding of drugs during pregnancy
- ☐ Increased renal clearance during pregnancy
- ☐ Delayed gastric emptying during pregnancy
- ☐ Increased seizure threshold in pregnancy

Your answer is incorrect.

Explanation:

Pregnancy results in delayed gastric emptying, which can have an impact on the risk of aspiration during ECT. When assessing a pregnant patient for ECT, the anaesthetist will consider risks of the anaesthetic agents used; positioning of the patient to take pressure off the large vessels of the abdomen and the risk of aspiration, given the delayed gastric emptying. An obstetric review should identify women with high obstetric risks, particularly of vaginal bleeding or preterm labour.

Ref: Ferrier IN, Waite J, editors. The ECT handbook. Cambridge University Press; 2019. p. 64.

The correct answer is: Delayed gastric emptying during pregnancy

Question 33

Not answered

Marked out of 1

What is the risk of neonatal spina bifida when a woman is treated with carbamazepine during pregnancy?

Select one:

- ☐ 1-2%
- ☐ 0.3-0.5%
- ☐ 20%
- ☐ 10%
- ☐ 3-5%

Your answer is incorrect.

Explanation:

Both carbamazepine and valproate have a clear causal link with increased risk of a variety of foetal abnormalities, particularly neural tube defects including spina bifida. Valproate confers a higher risk than carbamazepine. Early pregnancy exposure to carbamazepine is linked with an approximately two-fold increased rate of congenital malformation (from around 2% to approximately 3.5-5%) and an approximate three-fold increased risk of spina bifida (from approximately 0.09% in the general population to approximately 0.3%).

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 690-691.

McAllister-Williams RH et al. British Association for Psychopharmacology consensus guidance on the use of psychotropic medication preconception, in pregnancy and postpartum 2017. Journal of Psychopharmacology. 2017 May;31(5):519-52.

The correct answer is: 0.3-0.5%

Question **34**

Not answered

Marked out of 1

Mrs. X suffers from severe insomnia following childbirth. She has no features of clinical depression. Which sedative would be safe during breast feeding?

Select one:

- ☐ Zovirax
- ☐ Zorax
- ☐ Zaleplon
- ☐ Zopiclone
- ☐ Zolpidem

Your answer is incorrect.

Buspirone, zaleplon and zopiclone are excreted in breast milk and should be avoided. Zopiclone: Infant serum range - up to 50% of the maternal plasma level. Zolpidem is relatively safe during breast feeding.

The correct answer is: Zolpidem



Question 35

Not answered

Marked out of 1

Which of the following is NOT a significant risk for postnatal depression?

Select one:

- ☐ Delivery at home
- ☐ Serious health problems in the child
- ☐ Past history of depression
- ☐ History of complicated antenatal issues
- ☐ History of traumatic delivery

Your answer is incorrect.

Pop et al (1995) investigated this issue and found that there was no difference in the incidence of blues and depression between women who gave birth at home and those who gave birth in hospital. But other listed factors are known to be associated with depression in general and postnatal depression in particular. Ref: Blues and depression during the early puerperium: home versus hospital deliveries, Pop et al., Br J Obstet Gynaecol. 1995 Sep;102(9):701-6.

The correct answer is: Delivery at home

Question **36**

Not answered

Marked out of 1

Use of clonazepam in pregnancy is most likely to be associated with

Select one:

- ☐ Ventricular septal defect
- ☐ Neonatal hypotonia
- ☐ Ebstein anomaly
- ☐ Persistent pulmonary hypertension
- ☐ Intellectual disability

Your answer is incorrect.

Explanation:

First-trimester exposure to benzodiazepines was associated with an increased risk of oral clefts in newborns, but subsequent studies have failed to confirm this association. Benzodiazepines are not major human teratogens but should ideally be gradually discontinued before a planned pregnancy. Third trimester use is well known to cause 'floppy baby syndrome'/hypotonia.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 518, 535, 692.

The correct answer is: Neonatal hypotonia

Question **37**

Not answered

Marked out of 1

Which class of antidepressants is preferred for use in breastfeeding mothers?

Select one:

- ☐ NDRI
- ☐ SNRI
- ☐ NaSSA
- ☐ SSRI
- ☐ TCA

Your answer is incorrect.

Explanation:

The SSRI sertraline appears to have the most minimal passage into breastmilk and so is preferred when a woman is newly starting therapy. All SSRIs pass minimally into breastmilk at a level considered compatible with breastfeeding; rarely has a serious adverse event in an exposed healthy full-term infant. The clear benefits of breastfeeding in most cases outweigh concerns about SSRI exposure. When SSRIs are ineffective, women may be switched to other antidepressants, on which less information exists about lactation exposure. In general, SNRIs and mirtazapine (NaSSA) appear to have minimal passage into breastmilk, whereas bupropion (NDRI) is avoided if possible due to some case reports of infant seizure. Tricyclics (TCAs) have greater passage into breastmilk than SSRIs and so are avoided when possible; doxepin is considered contraindicated given case reports of adverse events in exposed infants.

Ref: Stewart DE, Vigod SN. Postpartum depression: pathophysiology, treatment, and emerging therapeutics. Annual review of medicine. 2019 Jan 27;70:183-96.

The correct answer is: SSRI

Question **38**

Not answered

Marked out of 1

Which of the following neurocognitive functions is most affected in children exposed to valproate during pregnancy?

Select one:

- ☐ Performance IQ
- ☐ Verbal IQ
- ☐ Procedural memory
- ☐ Executive function
- ☐ Attention span

Your answer is incorrect.

An increasingly recognised risk in exposed children is mental retardation-low IQ due to Valproate exposure seems to be the most common result of the mother being treated with Valproate when pregnant. Among children whose mothers took valproate monotherapy in pregnancy 42% had a verbal IQ score of less than 80, and 22% had <70. (Adab et al., 2004). Interestingly in epilepsy literature, it is also recorded that having 5 or more seizures during pregnancy can significantly affect the verbal IQ of the newborn. So the confounding effect of seizures is not known. They also reported that 30% of children exposed to valproate needed special educational support in school, compared with 3-6% of those exposed to monotherapy with other antiepileptic drugs (Breen and amp; Davenport, 2006)

The correct answer is: Verbal IQ

Question 39

Not answered

Marked out of 1

A GP refers a young woman suffering from premenstrual dysphoric disorder. She has tried primrose oil, calcium supplements and exercise regimes without benefit. You decide to treat her with sertraline. Which of the following is correct?

Select one:

- ☐ Intermittent use of sertraline can be suggested
- ☐ Sertraline takes at least 3 menstrual cycles to show any effect
- ☐ Combination therapy with thyroxine can be suggested
- ☐ She must have clinical depression to respond to sertraline
- ☐ Sertraline must be used at a higher than antidepressant dose

Your answer is incorrect.

Explanation:

Once the diagnosis of premenstrual dysphoric disorder (PMDD) is confirmed, an SSRI is considered first-line treatment. SSRIs may be effective when administered daily or intermittently (from ovulation to menses or during symptomatic days). A significant body of evidence supports the use of SSRIs dosed either continuously or during the luteal phase as an effective first-line treatment for PMDD.

Ref: di Scalea TL, Pearlstein T. Premenstrual dysphoric disorder. Med Clin North Am. 2019 Jul;103(4):613-628.

Carlini SV, Deligiannidis KM. Evidence-based treatment of premenstrual dysphoric disorder: a concise review. The Journal of clinical psychiatry. 2020 Feb 4;81(2):6789.

The correct answer is: Intermittent use of sertraline can be suggested

Question 40

Not answered

Marked out of 1

Intermittent dosing of SSRIs in the treatment of severe PMS/PMDD is

Select one:

- ☐ Poorly tolerated
- ☐ More effective than continuous dosing
- ☐ Associated with significant discontinuation symptoms
- ☐ More expensive than continuous dosing
- ☐ Less effective for physical symptoms

Your answer is incorrect.

Explanation:

A significant body of evidence supports the use of SSRIs dosed either continuously or during the luteal phase as an effective first-line treatment for PMDD. Intermittent dosing of SSRIs is shown to be well-tolerated and not associated with significant discontinuation symptoms compared to placebo. Intermittent dosing may be less effective than continuous dosing for premenstrual physical symptoms.

Ref: Carlini SV, Deligiannidis KM. Evidence-based treatment of premenstrual dysphoric disorder: a concise review. The Journal of clinical psychiatry. 2020 Feb 4;81(2):6789.

di Scalea TL, Pearlstein T. Premenstrual dysphoric disorder. Med Clin North Am. 2019 Jul;103(4):613-628.

The correct answer is: Less effective for physical symptoms



Question **41**

Not answered

Marked out of 1

The prevalence of Ebstein anomaly in the general population is

Select one:

- ☐ 1 in 20 000
- ☐ 1 in 1000
- ☐ 1 in 100 000
- ☐ 1 in 2000
- ☐ 1 in 10 000

Your answer is incorrect.

Explanation:

There is an increased risk of Ebstein anomaly (cardiac malformation with prevalence in the general population of 1:20 000) if lithium taken during the 1st trimester of pregnancy (maximum risk 2-6 weeks after conception), though the size of the risk is uncertain. More recent data suggest that the magnitude of the effect is much smaller than previously estimated.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 690-692.

McAllister-Williams RH et al. British Association for Psychopharmacology consensus guidance on the use of psychotropic medication preconception, in pregnancy and postpartum 2017. Journal of Psychopharmacology. 2017 May;31(5):519-52.

The correct answer is: 1 in 20 000

Question 42

Not answered

Marked out of 1

A 32-year-old woman is pregnant for the first time. She is known to have bipolar disorder but has not taken any treatment for the last 4 months as she was trying to be pregnant. If she continues to be medication-free, which of the following is most likely to happen?

Select one:

- ☐ No relapse throughout pregnancy or postpartum
- ☐ Relapse into mania after delivery
- ☐ Relapse into depression after delivery
- ☐ Relapse into mania during pregnancy
- ☐ Relapse into depression during pregnancy

Your answer is incorrect.

Explanation:

Women with a history of bipolar disorder are at high risk for postpartum relapse. There is a greater probability of suffering episodes of postpartum psychosis/mania in women with a previous diagnosis of bipolar I disorder. Considering all types of bipolar disorder, depression is the most prevalent form of postpartum relapse.

Ref: Conejo-Galindo J et al. Postpartum Relapse in Patients with Bipolar Disorder. Journal of Clinical Medicine. 2022 Jul 8;11(14):3979.

The correct answer is: Relapse into depression after delivery



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